

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Nov 21, 2025

with Gregory Krause, MD at Mass Eye and Ear Otolaryngology Clinic



Notes from Care Team



Progress Notes by Gregory Krause, MD at 11/21/2025 1:30 PM



Mass General Brigham
Mass Eye and Ear



HARVARD
MEDICAL SCHOOL

MASSACHUSETTS EYE AND EAR INFIRMARY DEPARTMENT--OTOLARYNGOLOGY

Date: 11/21/2025

Patient: Daniel S May DOB: 10/11/1989 MEEI MRN: 8026121

PCP: Pcp, Not Required

History of Present Illness

Daniel S May is a 36 year old male who presents with persistent left ear fullness and drainage following ear tube placement.

He has persistent issues with his left ear, which feels blocked and has a yellowish-greenish discharge. He uses a Q-tip to clean a small amount of discharge in the morning.

Left PE tube was placed three weeks ago. Initially, he felt a slight improvement, describing it as being able to 'breathe a little bit more,' but no substantial relief was noted. About a week after the procedure, he began experiencing drainage from the ear.

He uses budesonide nasal rinse, which helps his nasal symptoms but does not seem to affect his bilateral aural fullness. No allergies are reported.

His right ear also feels more full, but he describes the symptoms as variable day-to-day. He notes that his sinuses have been good.

10/31/25 office visit-

Chronic left Eustachian tube dysfunction

He experiences significant pressure in the left ear, causing fullness, popping, and cracking, which have persisted for several months, affecting daily life and sleep. Previous treatments with nasal sprays and steroids were ineffective. An audiogram confirms Eustachian tube dysfunction. Due to chronicity and lack of response to medical management, tympanostomy tube placement was performed in the left ear. He was advised to keep water out of his ears for a few weeks post-procedure. After a couple weeks, swimming is allowed with the tube, but scuba diving is not. Potential risks of tympanostomy tube placement were discussed.

Chronic rhinosinusitis

Mr. May is being followed by Dr. Gray for his chronic sinusitis. He states he has been improved with use of budesonide rinses. He is out of refills. Additional refills prescribed.

EXAMINATION

There were no vitals taken for this visit.

GENERAL APPEARANCE: well developed, well nourished patient in no acute distress.

VOICE: normal

BREATHING: normal, not labored, no stridor

FACE: no lesions

FACIAL STRENGTH: normal and symmetric

AD: clear

AS: purulent discharge in EAC

Procedure: Examination of the Ear Under the Microscope

In order to further assess findings identified on otoscopy, binocular microscopy was performed.

Procedure discussed with patient and verbal consent obtained.

Patient placed under the microscope

AS: purulent discharge in EAC. Suctioned. Petube in place

Well tolerated, no bleeding or pain.

Assessment & Plan

Left ear otorrhea with infection post-tympanostomy tube

Persistent left ear otorrhea with yellowish-green discharge has occurred following tympanostomy tube placement three weeks ago. Initial improvement was noted, but drainage began approximately one week post-procedure. The tube is correctly positioned but associated with infection, likely causing fullness and discomfort. Prescribe Floxin ear drops for drainage. Advise keeping water out of the ear and using a cotton ball during showers. Schedule follow-up in two weeks to assess the need for further cleaning.

Chronic left Eustachian tube dysfunction

Chronic dysfunction with more pronounced negative pressure in the left ear. Recent tympanostomy tube placement aimed to alleviate symptoms, but drainage complicates assessment of efficacy. Continue to monitor response to ear drops and drainage management.

Right Eustachian tube dysfunction (suspected)

Suspected dysfunction with symptoms of fullness and discomfort. Delay intervention until the left ear condition is resolved and the efficacy of the tympanostomy tube is confirmed. Continue to monitor symptoms and reassess after the left ear condition improves.

Gregory E. Krause, MD

Comprehensive Otolaryngology

Massachusetts Eye and Ear Infirmary

Harvard Medical School